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1 Section 357. A. As used in Sections 357 through ~~360~~ 360.1 of
2 this title:

3 1. "Clean claim" means a claim that is submitted in accordance
4 with all applicable billing requirements, contains all information
5 reasonably necessary for adjudication, and is not subject to an
6 unresolved eligibility or coverage dispute at the time of
7 submission;

8 2. "Claim" means a request for payment or reimbursement
9 submitted by a provider for prescription drugs, pharmacy-dispensed
10 medical supplies or devices, professional pharmacy services, or
11 manufacturer coupon, copay-assistance, discount card, or other
12 similar transactions;

13 3. "Covered entity" means a nonprofit hospital or medical
14 service organization, for-profit hospital or medical service
15 organization, insurer, health benefit plan, health maintenance
16 organization, health program administered by the state in the
17 capacity of providing health coverage, or an employer, labor union,
18 or other group of persons that provides health coverage to persons
19 in this state. This term does not include a health benefit plan
20 that provides coverage only for accidental injury, specified
21 disease, hospital indemnity, disability income, or other limited
22 benefit health insurance policies and contracts that do not include
23 prescription drug coverage;

1 ~~2.~~ 4. "Covered individual" means a member, participant,
2 enrollee, contract holder or policy holder or beneficiary of a
3 covered entity who is provided health coverage by the covered
4 entity. A covered individual includes any dependent or other person
5 provided health coverage through a policy, contract or plan for a
6 covered individual;

7 ~~3.~~ 5. "Department" means the Insurance Department;

8 ~~4.~~ 6. "Effective rate contracting" means any agreement or
9 arrangement between a pharmacy or contracting agent acting on behalf
10 of a pharmacy and a pharmacy benefits manager for pharmaceuticals
11 based on the effective rate of payment rather than a predetermined
12 fixed price or fixed discount percentage;

13 ~~5.~~ 7. "Maximum allowable cost", "MAC", or "MAC list" means the
14 list of drug products delineating the maximum per-unit reimbursement
15 for multiple-source prescription drugs, medical ~~product~~ products, or
16 ~~device~~ devices;

17 ~~6.~~ 8. "Multisource drug product reimbursement" (reimbursement)
18 means the total amount paid to a pharmacy inclusive of any reduction
19 in payment to the pharmacy, excluding prescription dispense fees and
20 professional fees;

21 ~~7.~~ 9. "Office" means the Office of the Attorney General;

22 ~~8.~~ 10. "Payor" means any person or entity that adjudicates
23 processes, administers, controls, or funds payment or reimbursement
24 of a pharmacy claim including, but not limited to:

- a. pharmacy benefits managers,
- b. health insurers,
- c. health maintenance organizations,
- d. third-party administrators,
- e. self-funded or fully insured health benefit plans,
- f. government health programs,
- g. manufacturer coupon card, copay-assistance, or patient
assistance programs,
- h. discount card, voucher, rebate, or similar program
administrators, and
- i. any affiliate, agent, or contractor acting on behalf
of an entity provided in this paragraph.

Payor does not include a covered individual;

11. "Pharmacy benefits management" means a service provided to covered entities to facilitate the provision of prescription drug benefits to covered individuals within the state, including negotiating pricing and other terms with drug manufacturers and providers. Pharmacy benefits management may include any or all of the following services:

- a. claims processing, retail network management and payment of claims to pharmacies for prescription drugs dispensed to covered individuals,
- b. clinical formulary development and management services, or

1 c. rebate contracting and administration;

2 ~~9.~~ 12. "Pharmacy benefits manager" or "PBM" means a person,
3 business, or other entity that performs pharmacy benefits
4 management. The term shall include a person or entity acting on
5 behalf of a PBM in a contractual or employment relationship in the
6 performance of pharmacy benefits management for a managed care
7 company, nonprofit hospital, medical service organization, insurance
8 company, third-party payor, or a health program administered by an
9 agency or department of this state;

10 ~~10.~~ 13. "Plan sponsor" means the employers, insurance
11 companies, unions and health maintenance organizations or any other
12 entity responsible for establishing, maintaining, or administering a
13 health benefit plan on behalf of covered individuals; ~~and~~

14 ~~11.~~ 14. "Provider" means a pharmacy licensed by the State Board
15 of Pharmacy, or an agent or representative of a pharmacy, including,
16 but not limited to, the pharmacy's contracting agent, which
17 dispenses prescription drugs or devices to covered individuals; and

18 15. "Receipt" means the date on which a pharmacy claim is first
19 received by a payor or any agent of the payor, regardless of
20 internal routing or processing.

21 B. Nothing in the definition of pharmacy benefits management or
22 pharmacy benefits manager in the Patient's Right to Pharmacy Choice
23 Act, Pharmacy Audit Integrity Act, or Sections 357 through ~~360~~ 360.1
24 of this title shall deem ~~an~~ an:

1 1. An employer a ~~"pharmacy benefits manager"~~ pharmacy benefits
2 manager of its own self-funded health benefit plan, except, to the
3 extent permitted by applicable law, where the employer, without the
4 utilization of a third party and unrelated to the employer's own
5 pharmacy:

- 6 a. negotiates directly with drug manufacturers,
- 7 b. processes claims on behalf of its members, or
- 8 c. manages its own retail network of pharmacies; or

9 2. A covered entity or plan sponsor a pharmacy benefits manager
10 solely because such entity or sponsor retains ultimate authority to
11 establish, adopt, modify, or approve its own prescription drug
12 formulary, including utilization management criteria or negotiating
13 directly with drug manufacturers, while contracting with a pharmacy
14 benefits manager, third-party administrator, or other vendor to
15 administer, process, adjudicate, or pay pharmacy claims on the
16 entity's or sponsor's behalf, provided that:

- 17 a. the covered entity or plan sponsor retains final
18 decision-making authority over formulary design and
19 formulary changes including, but not limited to, the
20 power to reject any addition, removal, tiering change,
21 prior authorization requirement, step therapy
22 requirement, or other utilization management
23 requirement, and

1 b. the pharmacy benefits manager or other vendor acts in
2 an administrative capacity only with respect to claims
3 processing, adjudication, pharmacy network
4 administration, and payment functions, and does not
5 possess unilateral authority to change the formulary
6 other than implementing changes authorized by the
7 covered entity or plan sponsor.

8 SECTION 2. NEW LAW A new section of law to be codified
9 in the Oklahoma Statutes as Section 360.2 of Title 59, unless there
10 is created a duplication in numbering, reads as follows:

11 A. Payment to a provider for a claim shall not be conditioned
12 upon post-transaction reconciliation, manufacturer funding cycles,
13 or internal settlement between program sponsors, administrators, or
14 affiliates.

15 B. A provider shall not bear the risk of delayed or failed
16 funding between a manufacturer, administrator, or other third party
17 after a claim is adjudicated or accepted at the point of sale.

18 C. A payor shall remit full payment for a clean claim no later
19 than thirty (30) calendar days after the receipt of the clean claim.
20 Nothing in this subsection shall prohibit or discourage payment in a
21 shorter time period, including expedited payment of electronically
22 submitted claims.

23 D. A payor shall provide a provider with a clear, accurate, and
24 individualized accounting of all payments made to the provider for

1 claims. The accounting shall be provided in a readable, itemized
2 format, including electronic remittance advice or other electronic
3 format commonly used in the pharmacy industry, and shall not require
4 the provider to aggregate, infer, or reconstruct claim-level payment
5 information. Such accounting shall be provided with each payment or
6 remittance and shall be presented at a claim-by-claim level that
7 reasonably allows the provider to identify:

- 8 1. A unique claim identifier or prescription number;
- 9 2. The date of service or dispensing date;
- 10 3. The total amount paid for the claim by the payor;
- 11 4. The total amount paid for the claim by the covered
12 individual or plan member;
- 13 5. The total amount paid to the pharmacy for reimbursement;
- 14 6. Any amounts withheld, reduced, or adjusted, including the
15 reason for such adjustment;
- 16 7. Any fees, assessments, or offsets applied to the claim;
- 17 8. The identity of the payor or program responsible for the
18 payment, including identification of any manufacturer coupon, copay-
19 assistance, or discount card program involved;
- 20 9. The final payment date for the claim; and
- 21 10. Any interest paid on a claim pursuant to subsection F of
22 this section.

23 E. A payor shall not:

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- 1 1. Bundle or net multiple claims in a manner that obscures
2 claim-level payment information;
- 3 2. Provide only summary, aggregate, or plan-level payment data
4 in lieu of individualized claim accounting;
- 5 3. Condition access to individualized claim accounting on
6 additional fees, portal subscriptions, or contractual waivers;
- 7 4. Delay payment of an adjudicated or accepted claim beyond the
8 time frames established pursuant to subsection C of this section;
- 9 5. Retroactively reprice, reverse, or withhold payment after
10 adjudication, except as otherwise expressly permitted by state law;
- 11 6. Condition or withhold payment based on audits conducted
12 after adjudication;
- 13 7. Extend payment timelines through contract, policy, program
14 terms, or operating rules inconsistent with subsection C of this
15 section; or
- 16 8. Shift payment risk to a provider due to internal disputes,
17 funding delays, or administrative issues of the payor or the payor's
18 affiliates.

19 F. Any payment not made within the time frame set forth in
20 subsection C of this section shall automatically accrue interest
21 beginning on the day after the expiration of such time frame. Such
22 interest shall accrue at a rate of ten percent (10%) per thirty (30)
23 days, calculated solely on the unpaid amount owed by the payor to
24 the provider. Interest assessed pursuant to this subsection shall

1 be non-waivable and shall be paid in addition to the underlying
2 claim amount.

3 G. A payor may be subject to any fines, penalties, and remedies
4 provided by state law. The Attorney General may levy a civil or
5 administrative fine not less than One Hundred Dollars (\$100.00) and
6 not more than Ten Thousand Dollars (\$10,000.00) per each violation
7 of this act.

8 H. Any contract, agreement, policy, or program term that
9 waives, limits, or extends the rights or timelines established
10 pursuant to this act shall be void and unenforceable.

11 I. The Attorney General may promulgate any rules necessary to
12 enforce the provisions of this act.

13 J. This section shall be applicable to all claims paid on or
14 after the effective date of this act regardless of the date a
15 contract or program was executed or the payment methodology or
16 reimbursement model used by the payor.

17 SECTION 3. This act shall become effective November 1, 2026.

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19 COMMITTEE REPORT BY: COMMITTEE ON HEALTH AND HUMAN SERVICES
20 OVERSIGHT, dated 04/13/2026 - DO PASS.

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